INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Alectinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00401
Version 6

Continuous twice-daily capsules

Treatment stages

01 **Daily alectinib**
Twice daily with food

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Alectinib targets an abnormal ALK growth signal. After complete surgery the aim is to reduce the chance of cancer returning; in advanced cancer the aim is to control the cancer. Your team explains the individual aim, alternatives and uncertainty.

ALK INHIBITOR

Alectinib

Blocks an abnormal ALK growth signal

ALK signal drives cancer growth → Alectinib blocks ALK activity → Growth can slow or stop

Alectinib blocks the abnormal ALK protein that drives growth in ALK-positive cancer cells. It can act against cancer in the body and brain. It can also affect healthy tissues, so blood tests and symptom checks remain important.

Take with food. Keep blood tests and report muscle weakness, breathing changes, jaundice or dizziness promptly.

Understanding your treatment

This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

Your treatment aim

NCCP 00401 v6 · official regimen · EMA Alecensa product information, updated 31 March 2026

How long treatment lasts

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

Evidence for your setting

After complete surgery, ALINA found that alectinib delayed cancer recurrence compared with platinum chemotherapy; whether it improves overall survival remains uncertain. In untreated advanced cancer, ALEX showed better cancer control than crizotinib, but its final survival comparison did not establish a clear overall-survival difference. Separate studies showed tumour responses after crizotinib. These groups cannot predict your personal outcome or rank all current treatment choices.

ALINA primary analysis: resected ALK-positive non-small-cell lung cancer (NSCLC), 2024 · ALINA updated DFS, ESMO 2025 abstract 1787MO; cutoff 8 December 2024 · ALEX final OS analysis, 2025; cutoff 28 April 2025 · Ou et al: post-crizotinib single-arm phase II global study

Evidence differs after surgery and in advanced cancer

Studies describe groups in a particular setting. Ask which comparison applies to you.

The after-surgery study does not yet establish a survival benefit. Advanced-disease trials do not compare every modern ALK medicine. Group averages do not predict an individual result.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Intervals are reference guidance, not booked appointments. Ask who arranges tests, reviews results and authorises treatment.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00401 v6 · official regimen · EMA Alecensa product information, updated 31 March 2026

THE WHOLE TREATMENT JOURNEY

Alectinib monotherapy: treatment and checks

Your treatment pattern

Continuous twice-daily capsules

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ALK-positive non-small-cell lung cancer (NSCLC) · adjuvant or advanced disease

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First month	Months 2–3	From month 4	Completion / change
Alectinib NCCP 00401 · SMPC	Confirm setting and prescription	Twice daily with food	Continue daily if authorised	Continue with review	Adjuvant maximum 24 months; advanced duration depends on benefit/ toxicity
Liver tests NCCP 00401 · SMPC	Baseline liver profile	Every 2 weeks	Every 2 weeks	Every 4 weeks or as needed	Follow unresolved abnormalities
creatine phosphokinase (CPK) / muscles NCCP 00401 · SMPC	Baseline CPK	Every 2 weeks	As clinically indicated	As clinically indicated	Check recovery if held
Blood count / kidneys NCCP 00401 · SMPC	Baseline full blood count (FBC) and renal profile	Every 4 weeks	Every 4 weeks	Every 4 weeks	Follow anaemia and renal concerns

Care / check	Before starting	First month	Months 2–3	From month 4	Completion / change
Pulse / blood pressure / electrocardiogram (ECG) NCCP 00401 · SMPC	Baseline blood pressure (BP) and ECG	Agree early repeat ECG timing	Pulse/BP and ECG as needed	Repeat as clinically indicated	Confirm recovery and medicine review
Cancer and symptoms NCCP 00401 · SMPC	Confirm aim and disease assessment	Report new symptoms promptly	Review tolerability and adherence	Imaging per Consultant plan	Agree follow-up or next treatment

What each check is for

1 Confirm setting and baseline

Before starting

Adjuvant and advanced treatment have different endpoints.

Ask / act: Confirm indication, prescription, access and monitoring responsibilities.

2 Early liver and muscle checks

First month and first 3 months

Important abnormalities can be asymptomatic.

Ask / act: Keep blood tests and report new muscle/liver symptoms.

3 Ongoing reviews

Throughout treatment

Benefit, tolerance and medicine interactions can change.

Ask / act: Confirm tests, symptom review, supply and disease assessment.

4 Adjuvant completion or advanced change

At planned endpoint or earlier clinical change

A single duration does not fit all indications.

Ask / act: Agree completion/follow-up or another treatment decision.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Capsules, checks and next decisions

No dates or personal information are needed. Columns show sequence, not equal periods.

Treatment or check	Timing in this view
Start safely	**Before starting** Confirm ALK result, treatment setting, prescription, baseline tests and prior-crizotinib washout.
Active treatment	**First month → Months 2–3 → From month 4** Alectinib twice daily with food, with no planned rest week.
Liver monitoring	**First month → Months 2–3 → From month 4** Every 2 weeks for 3 months; then every 4 weeks or as needed.
Muscles, blood and heart	**First month → Months 2–3 → From month 4** Early creatine phosphokinase (CPK) checks; 4-week blood count/kidney tests; pulse/blood pressure (BP) and agreed electrocardiogram (ECG) review.
Completion or change	**Completion / change** Adjuvant maximum 24 months; advanced treatment until progression or unacceptable toxicity.

If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.

Keep reporting symptoms between visits. An agreed treatment hold takes priority over the daily schedule.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Before the first capsules

Confirm a practical plan with your team.

Baseline checks

Confirm ALK-positive NSCLC by a validated test; establish indication, staging/resection and prior-treatment history. FBC, renal and liver profile, CPK, blood pressure and ECG before treatment. Review all medicines, supplements, muscle/lung/cardiac symptoms, pregnancy, contraception and breastfeeding. For treatment after crizotinib allow a minimum 7-day washout. For adjuvant treatment confirm complete resection, high-risk criteria and ECOG 0–1.

Oral treatment and access

Confirm the prescription and supply route. Adjuvant use is listed in NCCP, but the printed reimbursement field is N/A; the team checks current access separately.

Taking alectinib

The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them.

Missed or vomited dose

If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.



Medicines and food

Check all prescribed, over-the-counter and herbal products with pharmacy. Ask specifically about medicines that slow the pulse and grapefruit/Seville oranges. Do not stop another prescribed medicine on your own.

Tests and symptom review

Blood-test results may change without symptoms.

Scheduled monitoring

Liver profile every 2 weeks for the first 3 months, then every 4 weeks or as clinically indicated. FBC and renal profile every 4 weeks. CPK every 2 weeks for the first month, then as clinically indicated, including muscle symptoms. Blood pressure and pulse as clinically indicated. NCCP says ECG prior to cycle 2, then as indicated; a continuous oral regimen has no defined cycle length, so agree the first repeat ECG appointment locally. Assess symptoms, interactions, adherence, current dose and treatment benefit; scans and disease monitoring follow the Consultant plan.

Symptoms between visits

Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test.

Daily living and planning

Raise practical concerns early.

Sun protection

Avoid prolonged sunlight during treatment and for at least 7 days after stopping. Use broad-spectrum UVA/UVB sunscreen and lip balm sun protection factor (SPF) 50 or higher.

Pregnancy, contraception and breastfeeding

Women who could become pregnant need highly effective contraception during treatment and for at least 5 weeks afterwards. Men with female partners who could become pregnant need highly effective contraception during treatment and for at least 3 months afterwards. Oral contraceptive effectiveness may be reduced: ask for a suitable method. Do not breastfeed during treatment. Discuss fertility before treatment and contact the team promptly if pregnancy occurs.

Future treatment decisions

Discuss benefit, side effects and priorities at reviews. After planned adjuvant completion, agree the ongoing lung-cancer follow-up plan.

Practical support

Anti-sickness medicines

Alectinib is minimal-to-low oral emetic risk. Treat nausea with prescribed as-required medicine if needed. Routine preventive anti-sickness treatment and infusion premedication are not required; your team selects a suitable rescue medicine.

Capsules, food and missed doses

The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.

Muscles, liver, blood and breathing

Keep liver, CPK and blood-count appointments. Muscle symptoms, jaundice, dark urine, unusual fatigue, cough or breathlessness need prompt review even between appointments.

Heart rate and other medicines

Tell the team about dizziness, fainting and medicines that slow the pulse. Check new prescriptions, supplements, grapefruit and Seville oranges with pharmacy; interactions may require closer monitoring rather than an automatic dose change.

Sun protection

Pregnancy, contraception and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Supply and visits

Request the next supply before capsules run out.

Alectinib itself does not require a line or infusion pump.

Questions to ask

Does taking capsules mean fewer important risks?

Capsules are cancer treatment and can cause serious side effects. Keep the planned tests and use the oncology contact plan for new symptoms.

Does having side effects show it is working?

No. Side effects do not reliably indicate cancer response; your team assesses benefit using the relevant follow-up plan.

Which blood tests happen early?

Liver tests are planned every 2 weeks for 3 months. CPK is checked every 2 weeks for the first month. Blood count and kidney tests are checked every 4 weeks; symptoms may prompt extra tests.

When is my next ECG?

NCCP asks for an ECG before its second treatment cycle, but this medicine is taken continuously. Ask the team to confirm the first repeat ECG appointment and later checks.

Can I change or restart my dose myself?

No. If alectinib is held or reduced, ask what needs to recover, when review happens and who will authorise restarting.

What should I say when calling?

Give the medicine name, current dose, last dose, symptoms and when they began. Explain whether you can drink, pass urine and manage usual activities. Do not delay calling to collect details.

Can I use this page to decide whether tests are safe?

No. Blood results, symptoms, current dose and treatment history need the prescriber's assessment. This guide does not authorise treatment.

Do I need to enter dates?

No. This map shows general timing. Your hospital arranges actual appointments.

Explain the plan back: Can you explain your treatment aim and duration, how to take capsules with food, what to do after a missed or vomited dose, and which symptoms need urgent contact?

Confirm your alectinib treatment setting This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Alectinib daily oral treatment

Active medicines: alectinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Alectinib is minimal-to-low oral emetic risk. Treat nausea with prescribed as-required medicine if needed. Routine preventive anti-sickness treatment and infusion premedication are not required; your team selects a suitable rescue medicine.

- The usual starting dose is 600 mg twice daily: four 150 mg capsules each time, taken with food. Use your own prescription, which may specify a lower dose. Swallow capsules whole; do not open or dissolve them. If a dose is missed, take it only if the next dose is more than 6 hours away. If the next dose is due within 6 hours, skip the missed dose. Do not take two doses together. If you vomit after a dose, take the next dose at its usual time; do not take an extra dose.
- Keep liver, creatine phosphokinase (CPK) and blood-count appointments. Muscle symptoms, jaundice, dark urine, unusual fatigue, cough or breathlessness need prompt review even between appointments.
- Tell the team about dizziness, fainting and medicines that slow the pulse. Check new prescriptions, supplements, grapefruit and Seville oranges with pharmacy; interactions may require closer monitoring rather than an automatic dose change.
- Avoid prolonged sunlight during treatment and for at least 7 days after stopping. Use broad-spectrum UVA/UVB sunscreen and lip balm sun protection factor (SPF) 50 or higher.
- Women who could become pregnant need highly effective contraception during treatment and for at least 5 weeks afterwards. Men with female partners who could become pregnant need highly effective contraception during treatment and for at least 3 months afterwards. Oral contraceptive effectiveness may be reduced: ask for a suitable method. Do not breastfeed during treatment. Discuss fertility before treatment and contact the team promptly if pregnancy occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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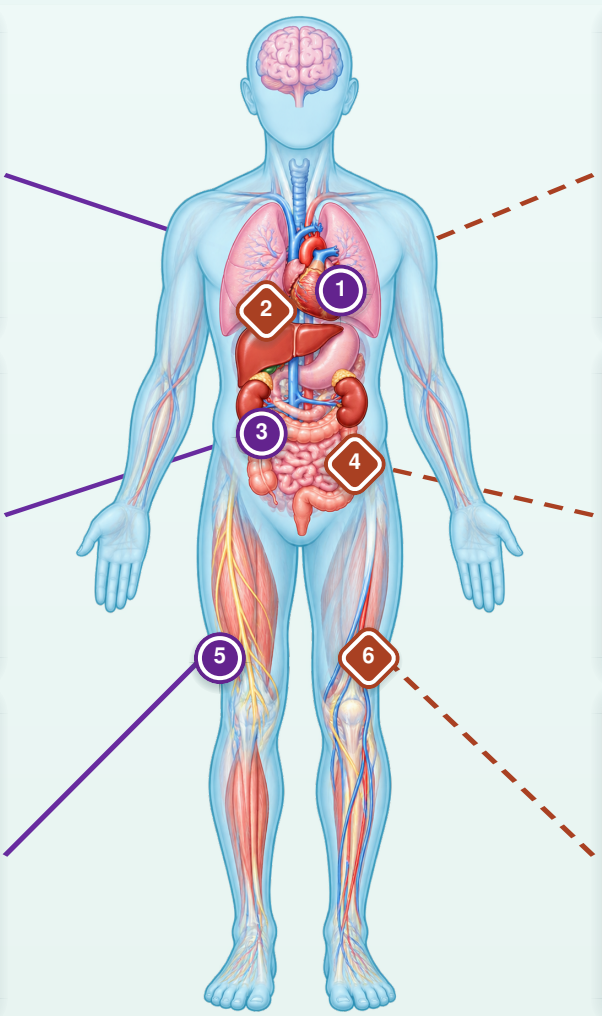
Alectinib monotherapy

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

ALK-targeted oral treatment · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

alectinib

Report promptly because lung inflammation can occur.

2 Dizziness or fainting

alectinib

A slow pulse can be symptomatic and needs medicine review.

3 Liver tests / jaundice

alectinib

Liver injury may start silently; keep tests and report yellow skin or dark urine.

4 Fatigue or pallor

alectinib

Anaemia can include red-cell breakdown; report marked changes.

5 Muscle pain or weakness

alectinib

creatine phosphokinase (CPK) tests help assess muscle injury; report unexplained symptoms.

6 Sun sensitivity

alectinib

Use high-factor sun protection during treatment and for 7 days after stopping.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Muscle pain / weakness	☐	☐	☐	
Breathlessness / cough	☐	☐	☐	
Dizziness / fainting	☐	☐	☐	
Constipation	☐	☐	☐	
Fatigue	☐	☐	☐	
Skin / sunlight changes	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test. Emergency symptoms: 112 / 999.

Carry your treatment details

Confirm your alectinib treatment setting This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Alectinib monotherapy

NCCP 00401 v6

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact the oncology team promptly for new or worsening cough or breathlessness, unexplained muscle pain or weakness, yellow skin or eyes, dark urine, marked tiredness or pallor, dizziness or fainting, severe abdominal pain, persistent vomiting or diarrhoea, or reduced urine. Sudden serious breathing difficulty, severe chest pain or collapse needs emergency help. Do not wait for the next blood test.

https://sactcheck.com/patient/00401/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00401 v6 · p4; oral classification Tables 5 and 8**
   https://healthservice.hse.ie/documents/6910/401_V6_Alectinib.pdf
2. **EMA Alecensa product information, updated 31 March 2026**
   https://www.ema.europa.eu/en/documents/product-information/alecensa-epar-product-information_en.pdf
3. **NCCP NCIS Medical Oncology antiemetics V8, February 2025**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **NCCP systemic anticancer therapy (SACT) nausea/vomiting classification V6, oral Tables 5 and 8**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
5. **ALINA primary analysis: resected ALK-positive non-small-cell lung cancer (NSCLC), 2024**
   https://pubmed.ncbi.nlm.nih.gov/38598794/
6. **ALINA updated DFS, ESMO 2025 abstract 1787MO; cutoff 8 December 2024**
   https://www.annalsofoncology.org/article/S0923-7534(25)03333-2/abstract
7. **ALEX final OS analysis, 2025; cutoff 28 April 2025**
   https://pubmed.ncbi.nlm.nih.gov/41110693/
8. **Ou et al: post-crizotinib single-arm phase II global study**
   https://pubmed.ncbi.nlm.nih.gov/26598747/